



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض رباعياً : نورا مهدي السيد اسماعيل
المحافظة : الجيزة
الحى / المدينة : سته اكتوبر
رقم البطاقة : 27902111302184 رقم التليفون : 01284506666
العمر (تاريخ الميلاد) : 11-02-1979 المهنة : ربه منزل
العنوان : -----
تاريخ تسجيل الإستمارة : 06-10-2026

الإقرار

أقر انا الموقع أدناه انني لا اتمتع باي نظام علاج مثل :-التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات او اي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك
الإخصائي الاجتماعي
التوقيع: (-----)
المقرر بما فيه
التوقيع: (-----)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : علاج مبادرة سرطان الثدي علاج هرموني خط ثاني مثبتات
الاروماتيز [14161]

التعليق على التشخيص : -----

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني
الاسم : د/ندى حسن
الاسم : د/نرمين رمضان
الوظيفة :
الوظيفة :
الوظيفة :
التوقيع :
التوقيع :
التوقيع :
أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية.
كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.
تعتمد صحة البيانات المرفقة
رئيس اللجنة ومدير
المستشفى.

تتعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

أ.د. /----- /د. /----- /د. /----- /د. /-----

قرار اللجنة

التشخيص النهائي :
خطة العلاج :
المستشفى :
قسم :
جهة العلاج :
نفقات :
ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

--

Name: نورا مهدي السيد اسماعيل National ID: 27902111302184	
--	---

تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بأن نوضح فيما يلي تقريراً عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
---	---

وبعد الكشف والفحوصات والمتابعة وجد أنه
--

المریضة تعاني من ورم غير حميد بالثدي و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطاقة تحقيق الشخصية

نورا
مهدي السيد اسماعيل المهدي
١٩٥٠ الحى الثامن - مع ١
٦ أكتوبر أول - الجيزة

٢٧٩٠٢١١٣٠٢١٨٤

KB7716118

٢٠٢٣/٠٧ ٢٧٩٠٢١١٣٠٢١٨٤

حاصلة على ليسانس ادب
قسم اجتماع
أنثى
مستلمة

البطاقة سارية حتى ٢٠٣٠/٠٧/١٦

١

Patient's Name: Nora Mahdi El-Sayed Ismail
Date: 2026-08-05
Referred by: Prof.Dr. no doctor
Hospital: Goustave Roussey International Egypt

**MULTISCLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST, ABDOMEN
AND PELVIS REVEALED:**

Known case of breast CA underwent right MRM with breast implant seen.

I- CHEST:

- Anterior mediastinal mass lesion is seen measuring 51 x 36 mm.
- Few mediastinal and right hilar lymph nodes largest measuring 22 x 18 mm at the right hilar region.
- Right lower lobe pleural based mass lesions measuring 42 x 30 mm.
- Small right lower lobe consolidation.
- Mild right nodular pleural thickening.
- No pleural or pericardial collection.
- Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- Average sized liver showing homogenous parenchymal attenuation. Left hepatic lobe segment IV enhancing area measuring 25 x 14 mm. No dilated biliary radicles.
- Both kidneys are normal size, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- Small left adrenal nodule measuring 9 x 6 mm for MR assessment.
- Normal C.T appearance of the spleen, pancreas, right adrenal gland, aorta and IVC.
- Left adnexal lesion measuring 42 x 39 mm.
- The urinary bladder shows no evidence of gross masses or diverticulae or stones.
- Normal appearance of the rest of pelvic organs.
- No abdominal or pelvic lymphadenopathy.
- No ascites.

Opinion:

- ❖ Two right lower lobe and anterior mediastinal mass lesions with few mediastinal and right hilar lymph nodes.
- ❖ Right lower lobe consolidation.
- ❖ Left hepatic lobe focal lesion for dynamic MR study.
- ❖ Left adnexal lesion for MR assessment.

Dr.sherif Wagdy

Name: نورا مهدي السيد اسماعيل		
MR. : 6414		
Detailed Medical Report		
		السيد
تحتية طبية وبعد,,		
	بشأن تقرير طبي للمريض	إيماء للطلب المقدم من
	رقم طبي:	بقسم:
ننشرف بأن نوضح فيما يلي تقريراً عن حالته		
	تاريخ وساعة الخروج :	تاريخ وساعة الدخول :
وبعد الكشف والفحوصات والمتابعة وجد انه		
The patient is a 47-year-old premenopausal individual with a clinical history of right breast cancer. Initial management in 2019 included six cycles of neoadjuvant FAC, followed by right mastectomy with reconstruction and postoperative radiotherapy. Adjuvant endocrine therapy with tamoxifen was administered for two years, with consideration for continuation. In 2024 the patient developed a pleural effusion; cytology demonstrated an inflammatory exudate with mesothelial cell proliferation, and a pigtail catheter was placed. A subsequent biopsy documented metastatic deposits of breast origin with estrogen receptor (ER) 7/8, progesterone receptor (PR) 3/8, HER2 negative, and Ki-67 20%. Imaging performed in August 2024 showed a mediastinal mass measuring 5.1 × 3.6 cm with progressive decrease in size, accompanied by new diffuse mediastinal and right hilar lymphadenopathy, the largest node measuring 2.2 × 1.8 cm. A CT-guided biopsy was requested to obtain tissue for definitive diagnosis. The patient was receiving hormonal therapy with Zoladex and Arimidex, and targeted therapy with Ibrance as of August 2024. The referring team recommended continuation of the current systemic regimen and tissue confirmation via CT-guided biopsy. The multidisciplinary team agreed with these recommendations and scheduled a re-MDT discussion after the biopsy results are available.		
تحريرا في ,		
	الوقت :	التاريخ : 23-09-2026
		الطبيب المعالج :
يعتمد ,		

Name : Nora Mahdi El-Sayed Ismail

Date : 07-05-2026 11:04

Branch : Dokki

Medical No. : 369397

Ref.Physician Prof : Goustav Roussey Egypt

Tc-99m MDP SKELETAL SCINTIGRAPHY

History

- 47-year-old female patient with history of breast cancer, complains of bone aches; referred for metastatic workup.

Technique

- Anterior and posterior whole-body scans were acquired 2 – 3 hours post injection of 20 mCi of Tc-99m MDP.

Findings

- The skeletal system demonstrates physiological radiotracer distribution with no focal abnormal areas of increased or decreased tracer uptake that would suggest skeletal metastatic deposits on scintigraphic basis.
- Degenerative changes at different levels of the spine and around big joints without suspicious metastatic pattern.

IMPRESSION

- **No Definitive Metastatic Disease:** There is no scintigraphic evidence of multifocal osteoblastic metastatic deposits in the surveyed skeleton at this time.
- Follow-up as per clinical pathway.

Much oblige
Dr. Alaa Nagi, MD
Nuclear medicine

10/07/2024, 15:15

code.42 - Name_A MDT form Harmal - Name_E MDT form Harmal



Presidential Program for Women's Health
Ministry of Health and Population

البرنامج الرئاسي لصحة المرأة
وزارة الصحة والسكان



A-Patient ID:

1- Name : نورا مهدي السيد اسماعيل

Sex:

MDT form

3- Medical number:
Female

83206224

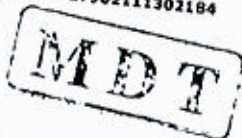
Old Med :

Old Med :

1478-2024

2-ID number: 27902111302184

Patient Age : 45 Y



Family history

Family History

Phone

Phone

01016759126

BMI

BMI

marital status

Marital Status

NRS Score

NRS Score

GROUP

GROUP

Co morbidity

Co morbidity

Menopausal
status

Menopausal Status

Initial STAGE

Initial STAGE

REASON OF MDT

REASON OF MDT

Date of MDT

Date of MDT

10/07/2024

Breast Imaging :

Clinical data

Mets Workup :

CT CAO WITH CONTRAST 5/2024: rt pleural polyprodal
thickening involving the mediastinal & costal pails
3 cm mas thickness

BX

IDC

BX COMMENT

ER

7/8

ER COMMENT

PR

PR COMMENT

-VE

Her 2 Neu

Her 2 Neu COMMENT

-VE

SISH

SISH COMMENT

KI67

18-20

T

N

M

1

metastatic to
pleura + pleural effusion

Sonomammography

PATIENT HISTORY

rt mastectomy & reconstruction 2019 after NACT (FEC X6)--->PORT -->zoladex / tam x 2 yrs
tam done
pleural effusion -->big tail
bone scan: free
pt on aramdix + zoladex

NOT OUTCOME & PLAN

MDT OUTCOME & PLAN

IN VIEW OF ENDOCRINE RESISTANCE
FOR ZOLADEX/AI + CDK4/6 INHIBITOR

MEDICAL ONCOLOGY

MEDICAL ONCOLOGY

Signature

Signature

Signature

Signature

Signature

Signature

RADIOLOGY

PATHOLOGY

Nutrition

RADIOLOGY

Signature

Signature

Signature

Signature

Signature

Patient Name : الأستاذة / نورا مهدي السيد اسماعيل
Patient No. : 27902111302184
Path No. : 1672/24
Sex : Female
Age : 45 Y
Referring Doc :

Request No. : 13000221
Request Date : 22/05/2024
Verification Date : 09/06/2024
Financial Category : اورام دار السلام - نفقة الدولة
Department : Out Patient
Registered by : Mrs. Mayada Hegazy

Pathology Report

Clinical Data: Plural & pulmonary lesions, known case of breast cancer.

Nature of Specimen: Tru-cut biopsy, collected at 21/5/2024, 01:05 pm.

Gross Examination:

- Multiple grayish white tissue cores up to 0.7 cm, totally submitted.

Microscopic Examination:

* Revealed plural & pulmonary tissue showed infiltration by malignant growth formed of malignant epithelial hyperchromatic pleomorphic cells.

IPT:

GATA3: Positive.

Diagnosis:

Plural & pulmonary lesions, Tru-cut biopsy:

Positive metastatic deposits of primary breast origin.

For further IHC (ER, PR, Her2, Ki67).

Signature
Prof. Dr. Lubna Omar.
Dr. Menna Nabil.

Verified by :

Printing Date : 22/06/2024 13:10:14

Page 1 of 1

Printing By : Mrs. Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name	: الأستاذة / نورا مهدى السيد اسماعيل	Request No.	: 13000221
Patient No.	: 27902111302184	Request Date	: 22/05/2024
Path No.	: 1672/24	Verification Date	: 22/06/2024
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 45 Y	Department	: Out Patient
Referring Doc. :		IHC No.	:
		Registered by	: Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data: Pleural & pulmonary lesions, in known case of breast cancer.

Nature of Specimen: Tru-cut biopsy, collected at 21/05/2024, 01:05 pm.

Technique:

- Sections were prepared from the paraffin block, processed in Bench Mark ULTRA machine (Ventana) and stained by rabbit monoclonal antibodies using ultra view HRP DAB as chromogen and hematoxylin as counter stain. The sections treated against:
- ER clone (1E2), PR clone (SP1), Her2 clone (4B5), & Ki67 clone (30-9), GATA-3 clone (L50-823).

Results:

- * IPT (Positive control showed appropriate reactivity):
- GATA 3: Positive in tumor cells.
- ER: Moderate nuclear reaction in 70 % of tumor cells (score 7/8).
- PR: Mild nuclear reaction in 3 % of tumor cells (score 3/8).
- Her2: Negative membrane reaction in tumor cells (score 0).
- Ki67: 20 % nuclear reaction of tumor cells.

Conclusion:

Pleural & pulmonary lesions, Tru-cut biopsy:

Findings are compatible with metastatic carcinoma, of breast origin.

IPT:

- GATA 3; Positive.
- ER: Positive (score 7/8).
- PR: Positive (score 3/8).
- Her2: Negative (score 0).
- Ki67: Positive in 20 % of tumor cells.

Verified by :

Printing Date : 22/06/2024 13:10:15

Page 1 of 2

Printed By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

Patient Name الأستاذة / نورا مهدى السيد اسماعيل
Patient No. 27902111302184
Path No. 1672/24
Sex Female
Age 45 Y
Referring Doc.

Request No. 13000221
Request Date 22/05/2024
Verification Date 22/06/2024
Financial Category اورام دار السلام - نفقة الدولة
Department Out Patient

Registered by Mrs.Mayada Hegazy

IMMUNOHISTOCHEMISTRY REPORT

* Scoring system: "ER, PR"

ASCO-CAP ER/PR Test Guideline Recommendations: proportion of cells staining (0, no staining; 1, >1%; 2, between 1% and 10%; 3, between 11% and 33%; 4, between 34% and 66% and 5, between 67%-100% of the cells staining). Intensity of positive tumor cells (0, none; 1, weak; 2, intermediate; and 3, strong). Total score from 0 to 8.

* Scoring system: "Her2"

Reference: ASCO-CAP Her2 Test Guideline Recommendations, 2013. IHC 0: as defined by no staining observed or membrane staining that is incomplete and is faint/barely perceptible and within < 10% of the invasive tumor cells. IHC 1+: as defined by incomplete membrane staining that is faint/barely perceptible and within > 10% of the invasive tumor cells. IHC 2+: based on circumferential membrane staining that is incomplete and/or weak/moderate and within > 10% of the invasive tumor cells or complete and circumferential membrane staining that is intense and within < 10% of the invasive tumor cells. IHC 3+: based on circumferential membrane staining that is incomplete, intense > 10% of tumor cells.

Signature
Prof. Dr. Lobna Omar.
Dr. Heba Hassan.

Verified by :

Printing Date : 22/06/2024 13:10:15

Page 2 of 2

Printed By : Mrs.Mayada Hegazy



غرفة رقم (4) - الدور الأول - مبنى رقم (987) - كورنيش النيل - مصر القديمة - القاهرة - مصر
Room No. (4) - First Floor-Building No. 987- Nile Corniche - Misr El Qadema - Cairo, Egypt

الممسوحة ضوئياً بـ CamScanner

Name : Nora Mahdi El-Sayed Ismail

Branch : Maadi

Ref.Physician Prof : Gostaf Roseh Hospital

Date : 14/07/2025 03:45

Medical No. : 150992

Ins.Company : التأمين الصحي

Tc^{99m} MDP SKELETAL SCINTIGRAPHY

History:

- A 46-year-old lady, with history of breast cancer, referred for metastatic work up.

Technique:

- Anterior and posterior whole body scans were acquired 2 hours post injection of the tracer.

Findings:

- No abnormal active or cold focal bony lesions could be seen over the surveyed skeleton to account for metastatic bone disease.
- Degenerative changes are seen at the spine and big joints.

Impression:

- The skeleton is scintigraphically free from metastases.

*Much obliged
Dr. Alaa Nagi, MD
Consultant of nuclear medicine*

NON CONTRAST MDCT SCAN OF THE CHEST, ABDOMEN & PELVIS

Compared with previous scan dated September 2024

Findings:

I- CHEST

- Right mastectomy with intact silicone implant.
- Decreased previously noted right pleural polypoidal thickening is seen involving the mediastinal, diaphragmatic and costal parts measuring 3 cm (previous 4.5 cm) maximum thickness with minimal effusion. decreased underlying partial lung collapse is seen.
- No sizable pulmonary nodules, masses or cavitary lesions.
- No CT evidence of enlarged hilar or mediastinal lymph nodes.
- No evidence of left pleural effusion.
- Normal cardiac size and shape. No pericardial effusion detected.

II- ABDOMEN & PELVIS:

- The liver is of average size with homogenous CT texture. No focal lesions.
- No intra-hepatic or extra-hepatic biliary radicle dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas and both supra renals.
- No abdominal lymph node enlargement seen.
- No ascites.

III- AXIAL SKELETON:

- No lytic or sclerotic bone lesions

OPINION:

- Right mastectomy with intact silicone implant.
- Decreased right pleural polypoidal thickening with minimal effusion and related lung collapse.
- No denovo lesion.
- No CT evidence of abdominal or osseous metastasis detected limited by the non contrast study.

Much obliged,

Dr. Mahmoud Moawad(MD)

Patient's Name:

Nora Mahdi El-Sayed Ismail



Date:

2026-08-05

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Rousey International Egypt

MULTISLICE POST CONTRAST C.T. EXAMINATION OF THE CHEST.

ABDOMEN AND PELVIS REVEALED:

Known case of breast CA underwent right MRM with breast implant seen.
COMPARE TO PREVIOUS STUDY DATED 14/4/2026, THE CURRENT
STUDY SHOWED

I- CHEST:

- o Progression in size of the anterior mediastinal mass lesion is seen measuring 51 x 36 mm compare to 29x20mm at the previous study.
- o Newly developed few mediastinal and right hilar lymph nodes largest measuring 22 x 18 mm at the right hilar region.
- o No appreciable changes as regading the right lower lobe pleural based mass lesions measuring 42 x 30 mm.
- o Small right lower lobe consolidation.
- o Mild right nodular pleural thickening.
- o No pleural or pericardial collection.
- o Normal plain CT appearance of the heart and great vessels.

II- ABDOMEN AND PELVIS:

- o Average sized liver showing homogenous parenchymal attenuation. Still seen left hepatic lobe segment IV enhancing area measuring 25 x 14 mm compare to 28x13mm at the previous study. No dilated biliary radicles.
- o Both kidneys are normal site, average sized and showing smooth regular cortical outline and preserved parenchymal thickness and function. No stones or back pressure changes.
- o Small left adrenal nodule measuring 9 x 6 mm for MR assessment.
- o Normal C.T appearance of the spleen, pancreas, right adrenal gland, aorta and IVC.
- o Left adnexal lesion measuring 42 x 39 mm for US correlation.

Patient's Name:

Nora Mahdi El-Sayed Ismail



Date:

2026-08-05

Referred by:

Prof.Dr. no doctor

Hospital:

Goustave Roussey International Egypt

- o The urinary bladder shows no evidence of gross masses or diverticulae or stones
- o Normal appearance of the rest of pelvic organs.
- o No abdominal or pelvic lymphadenopathy.
- o No ascites.

Opinion:

- ❖ Progressive course of the anterior mediastinal mass lesion& mediastinal lymphadenopathy
- ❖ Stationary course of left hepatic lobe focal lesion for dynamic MR study.

Dr.sherif Wagdy

Dr.sherif Wagdy